Fatal ventricular arrhythmia after initiation of cardexatine: a case report

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Journal of Synthetic Pharmacovigilance 2026; doi:10.9999/jsp.2026.0233

Abstract

We report a fatal ventricular arrhythmia occurring approximately four weeks after initiation of cardexatine in a patient with stage 3 chronic kidney disease. Renal impairment may prolong exposure and is a plausible contributory factor. The case is reported because of the outcome and the short interval from initiation.

Keywords: cardexatine, arrhythmia, fatal outcome, case report

Introduction

Cardexatine is licensed for chronic heart failure. Its elimination is partly renal, and the summary of product characteristics advises caution in impairment without specifying a dose adjustment. Fatal arrhythmia has not previously been described in the published literature.

Case Report

We report a 62-year-old male with a history of chronic kidney disease stage 3, who was referred to our department for assessment.

The patient had been prescribed Cardexatine 5 mg once daily (cardexatine mesilate, prolonged-release tablet) for chronic heart failure, commencing 11 January 2026. The route of administration was oral.

A 62-year-old male developed a ventricular arrhythmia which did not respond to resuscitation, beginning 6 February 2026. The reported term was fatal arrhythmia.

The event met the regulatory criteria for a serious adverse reaction (death).

The suspected medicine was withdrawn. At the time of writing the outcome was recorded as fatal.

A 62-year-old male with chronic kidney disease stage 3 was started on Cardexatine 5 mg once daily for chronic heart failure. He collapsed at home on 6 February 2026. Paramedics recorded a ventricular arrhythmia which did not respond to resuscitation. He was pronounced dead at the scene. A post-mortem has been requested but results are not yet available.

Discussion

Causality cannot be established from a single fatal case without post-mortem toxicology, which was not available at the time of writing. The temporal association and the absence of an alternative precipitant nonetheless justify reporting. Prescribers should consider closer monitoring during the first month of therapy in patients with reduced renal clearance.

Conclusion

A fatal outcome in temporal association with a recently initiated medicine warrants expedited reporting regardless of the strength of the causality assessment.

Acknowledgements

The authors thank the clinical pharmacy team at the participating centre for their assistance with record retrieval.

Funding

This work received no specific grant from any funding agency in the public, commercial or not-for-profit sectors.

Conflict of interest

The authors declare that they have no competing interests.

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